



(<https://learningcenter.hfsa.org/>).



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 43 of 117



A 22-year-old man with newly diagnosed hypertrophic cardiomyopathy is referred by his general cardiologist to your clinic for further evaluation. He is able to walk several blocks and climb 2 flights of stairs without limitation. He is disappointed that he can no longer participate in his competitive recreational soccer league. Upon further questioning, he denies any chest pain, orthopnea, paroxysmal nocturnal dyspnea, palpitations, lightheadedness or syncope. He does not recall any family members who have had sudden cardiac death, although his maternal grandfather died of unknown causes at age 70. On examination, his vital signs are blood pressure 120/80, heart rate 80, respiratory rate 22, and pulse oximetry of 94% on room air. His exam is notable for III/VI systolic murmur at the apex but otherwise unremarkable.

He had an echocardiogram performed 2 weeks ago which showed left ventricular systolic function of 45% without apical aneurysm, left ventricular end diastolic dimension of 55 mm, interventricular septum of 27 mm, and posterior wall of 19 mm. His resting left ventricular outflow tract gradient is 30 mmHg and increases to 50 mmHg with provocation. A cardiac MRI shows left ventricular ejection fraction of 42%, right ventricular ejection fraction of 50%. You order an ambulatory event monitor which shows some occasional premature ventricular contractions and one 8 second period of sustained ventricular tachycardia.

Which of the following is the best next step in managing this patient?

- A. Refer for alcohol septal ablation
-  B. Start mavacamten 5 mg daily
-  C. Refer for implantable cardioverter defibrillator placement
- D. Refer for septal myectomy

Commentary References

Testing Point: The board-certified AHFTC specialist should recognize indications for implantable cardioverter defibrillator placement in adult patients with hypertrophic cardiomyopathy.

Correct Answer: C. Refer for implantable cardioverter defibrillator placement

Rationale: According to the 2020 American College of Cardiology / American Heart Association Guideline for Hypertrophic Cardiomyopathy, ICD placement is recommended (Class I) in patients with prior cardiac arrest, ventricular fibrillation, or sustained ventricular tachycardia. ICD placement is also reasonable (Class IIa) in patients with major sudden cardiac death risk markers, including massive left ventricular hypertrophy (≥ 30 mm), unexplained syncope, apical aneurysm, family history of sudden cardiac death, or left ventricular ejection fraction $< 50\%$.

This patient has documented ventricular tachycardia on ambulatory monitoring and reduced left ventricular systolic function (LVEF 42–45%), both of which increase sudden cardiac death risk. Therefore, referral for ICD placement is the most appropriate next step using shared decision-making.

Answer B: Mavacamten is indicated for patients with symptomatic obstructive HCM despite guideline-directed medical therapy. This patient is asymptomatic and therefore is not an appropriate candidate.

Answers A and D: Septal reduction therapies, including alcohol septal ablation and septal myectomy, are reserved for patients with severe symptomatic LV outflow tract obstruction despite medical therapy. This patient does not have limiting symptoms.

Powered by Oasis.



(<https://www.oasis-lms.com/>).